

Global Tuberculosis Burden & Treatment Gaps

Trends, demographic disparities and geographic patterns

A decision-support dashboard for finding where burden and service-coverage gaps are concentrated.

ESTIMATED INCIDENT TB

10.55M

NOTIFIED CASES

8.33M

CALCULATED COVERAGE

78.9%

Project snapshot · 2024

WHO published rounded global headlines of 10.7M incident cases and 1.23M deaths; this dashboard aggregates the reproducible country-level extract.

[Student Name]

23 June 2026

The analytical problem

A gap can emerge between disease burden and people reached by services

Estimated incident TB

10.55M

Notified new & relapse

8.33M

Estimated notification difference

2.22M

A surveillance signal — not proof every person was untreated

Objective

Build one interactive tool that moves from description to prioritization.

Where is TB burden highest?

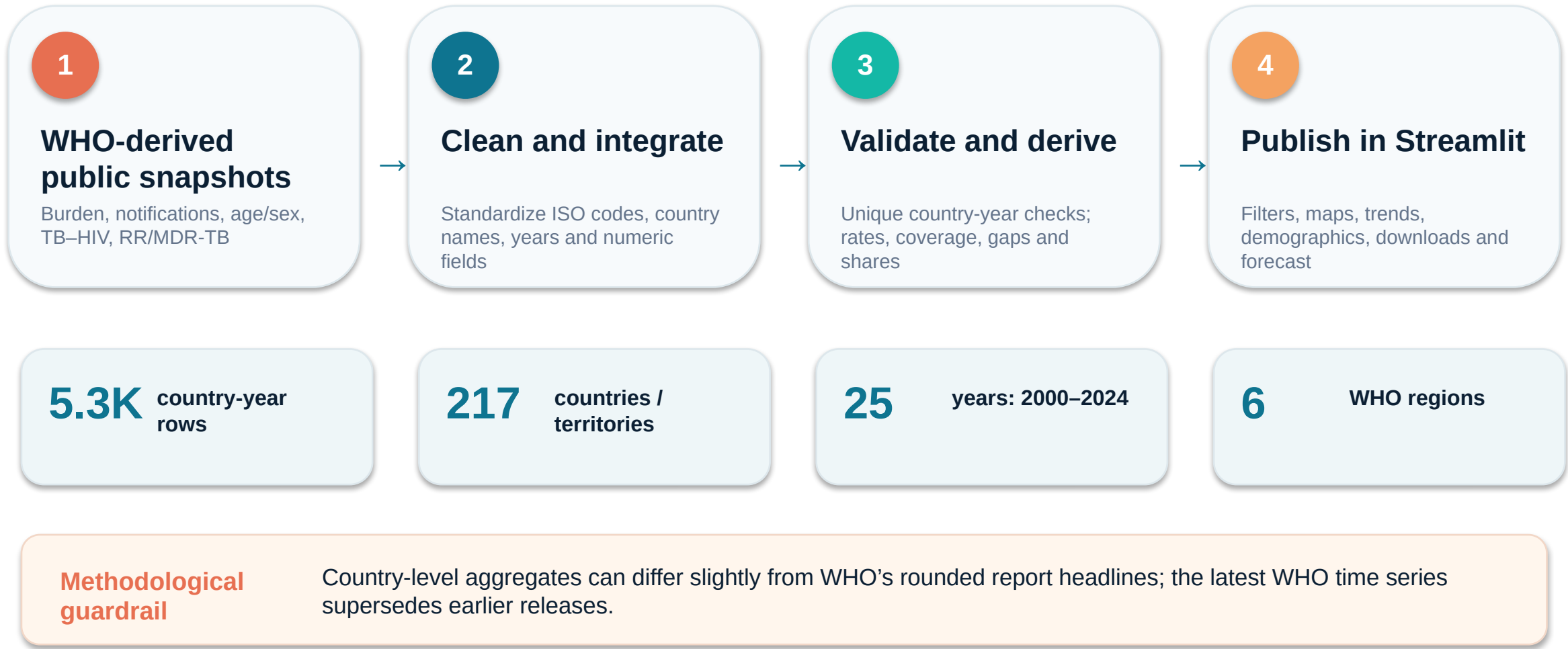
How is it changing over time?

Who and which subtypes are most affected?

Where should health systems investigate and act first?

Decision sequence: scale → geography → population → response

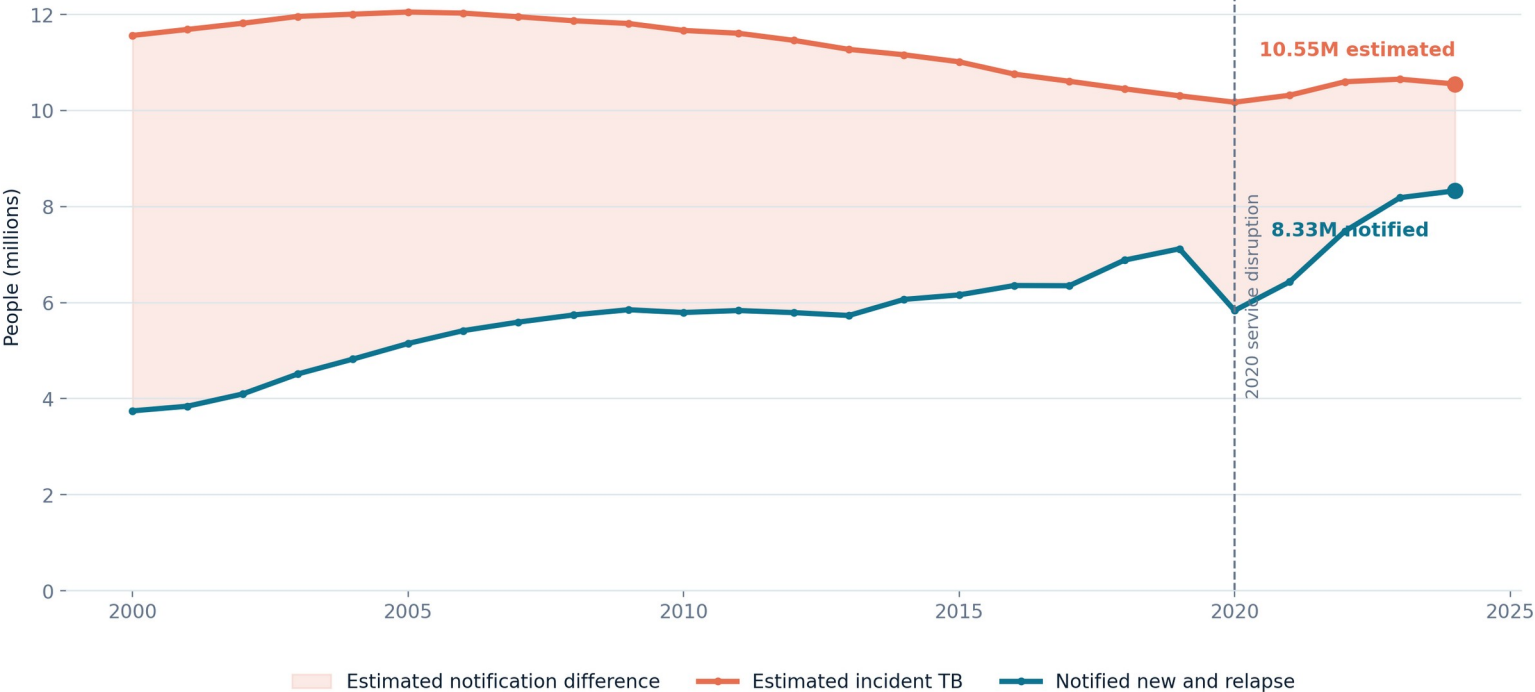
A reproducible, decision-ready data pipeline



Global progress is real — but the gap remains

Global TB burden and notified cases, 2000-2024

The gap narrowed substantially, but an estimated 2.22 million-case difference remained in the reproducible 2024 snapshot.



2024 SNAPSHOT

10.55M estimated
8.33M notified

REMAINING SIGNAL

2.22M notification
difference

SYSTEM SHOCK

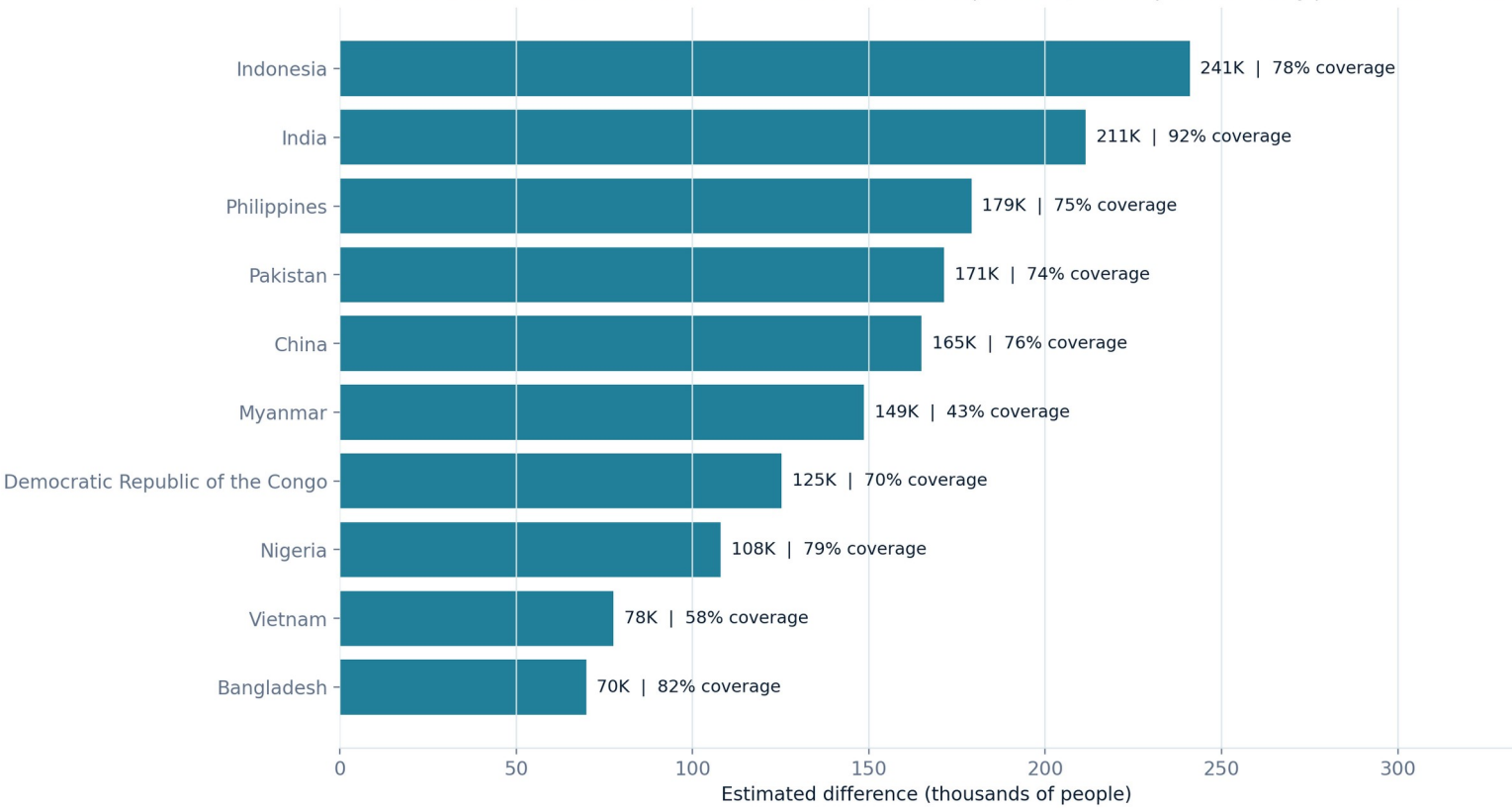
18.1% notification
fall in 2020

The gap narrowed, but detection and linkage remain incomplete.

Priority depends on burden volume and service coverage

Largest estimated notification differences, 2024

Difference = estimated incident cases minus notified new and relapse cases; it is not proof that every person was untreated.



Largest absolute difference

Indonesia · 241K



Largest total burden

**India · 2.71M
92% coverage**



Low coverage warning

Myanmar · 43.5%



Regional perspective

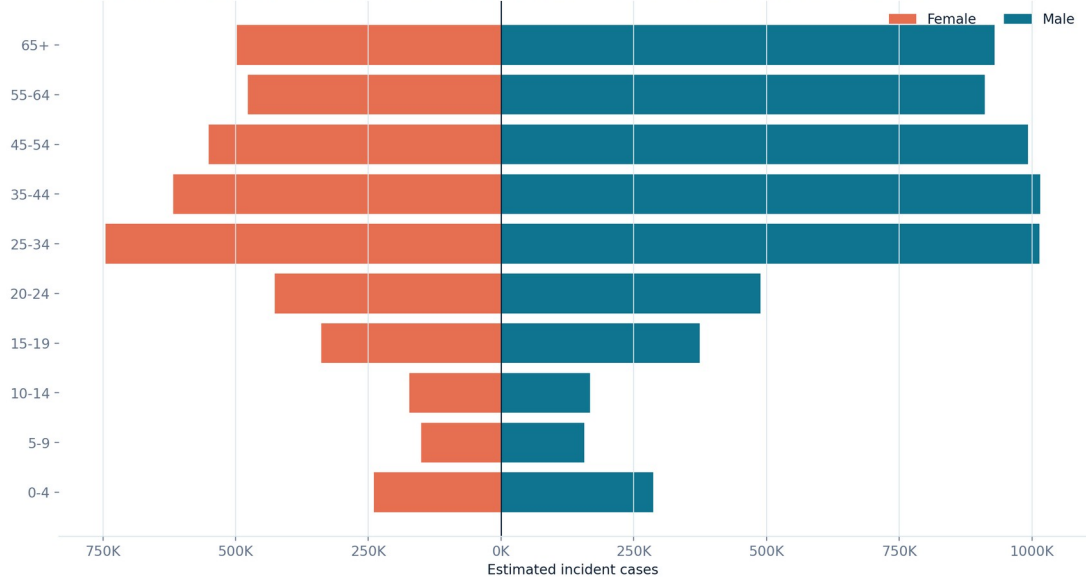
732K regional gap

**Use counts + rates + coverage
together.**

Different populations and clinical dimensions need different responses

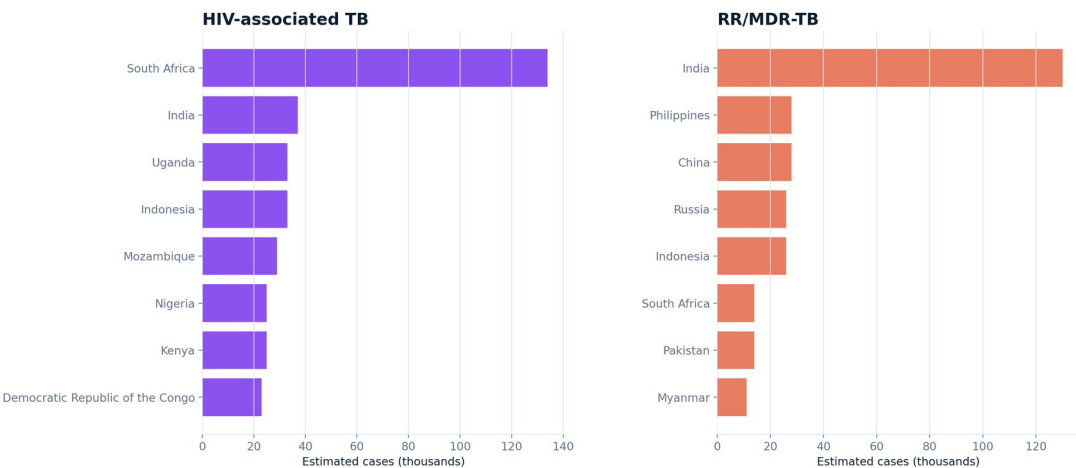
Estimated TB incidence by age and sex, 2024

The male burden exceeds the female burden in most adult age groups; total male-to-female ratio in this snapshot is about 1.50.



Two different high-risk dimensions require targeted responses

HIV weakens immunity and increases progression to active TB; RR/MDR-TB reflects resistance to key medicines.



Male : female ≈ 1.50
Tailor outreach to adult men

South Africa · 134K TB–HIV
Integrate TB and HIV pathways

India · 130K RR/MDR-TB
Expand rapid resistance testing

Four recommendations for a stronger TB response

1

Prioritize high-volume detection gaps

Focus case finding, diagnostics and reporting audits where absolute differences are largest.

2

Protect continuity during disruption

Maintain diagnostics, drug supply, follow-up and data reporting during emergencies.

3

Integrate TB–HIV and resistance pathways

Match the service model to co-epidemic and drug-resistance patterns.

4

Tailor outreach and govern the data

Use demographic targeting; investigate outliers and refresh with each WHO release.

FIND

where burden and gaps are concentrated



TARGET

the population and programme response



ACT

with measurable, data-governed interventions

Published dashboard URL: [PASTE STREAMLIT URL HERE]

Thank you